PT5053 – Combined Marking Framework (Quantitative & Qualitative)

Section	Weight (%)	80%+ (Excellent)	70–79% (Excellent)	60–69% (Good)	50–59% (Sound)	40–49% (Adequate)	30–39% (Inadequate)	≤29% (Poor)
Introduction & Background	5%	Demonstrates an exceptional depth and thorough grasp of the clinical context; precise aim/question; article fully identified; explicit use of relevant CASP; sets clear scope.	Demonstrates a thorough and detailed understanding f the topic; clear objectives; minor gaps; article and CASP identified.	Demonstrates an excellent understanding of the topic; clear objectives but limited theoretical integration; article and CASP identified.	Demonstrates a sound understanding of the topic; objectives present but unfocused; article and CASP identified.	Demonstrates an adequate understanding of the topic, but imprecise objectives; weak coherence; article and CASP identified.	Demonstrates an insufficient understanding of the topic; unclear objectives; little grasp; may omit aim/CASP.	Demonstrates a very poor understanding of the topic; no clear objectives or relevant content; no aim/question or CASP.
Methods: Study Design, Sampling & Recruitment, Data Collection Methods	30%	Provides an excellent accurate, detailed and rigorous evaluation of the study design and methods. Shows excellent alignment between the research aim/question and design. All sampling, recruitment, and data collection methods are identified accurately and analysed in depth with exceptional critical appraisal.	Provides a clear and accurate identification of the study design and methods. Critique is strong with only minor areas lacking depth. Demonstrates very good evaluation of sampling, recruitment, and data collection with minimal errors.	Identifies the study design and methods mostly accurately with some minor omissions or inconsistencies. Provides competent appraisal with reasonable integration of literature.	Shows a basic awareness of study design and methods, but accuracy may be partial or superficial. Some areas are described rather than critically evaluated; depth is limited; occasional inaccurate interpretation of sampling or data collection procedures.	Identification of study design/methods is often incomplete or imprecise. Evidence of misunderstanding or limited critique. Frequent gaps in evaluating sampling, recruitment, or data collection. Demonstrates limited accuracy in describing and interpreting methodological elements.	Inaccurate identification of the study design and key methods. Demonstrates weak understanding, incorrect interpretation of sampling or data collection, and poor critique. Referencing and source use are inaccurate or inappropriate.	Study design and methods are not accurately identified OR are identified incorrectly. Analysis is incorrect, irrelevant, or missing. Demonstrates no understanding of methodological principles. No meaningful appraisal of sampling, recruitment, or data collection.
Data Analysis & Findings; Strengths, Limitations & Trustworthiness	20%	Excellent transparent, rigorous analysis; credible findings; strengths/limitations critically appraised; trustworthiness well evidenced.	Clear analysis; credible findings; sound critique with minor gaps.	Reasonably clear analysis; some critical appraisal; limited depth.	General analysis; conclusions somewhat supported; superficial critique.	Limited clarity/credibility; minimal appraisal of trustworthiness.	Unclear analysis; weak linkage to conclusions; poor appraisal.	Absent/incorrect analysis; unsupported findings; no appraisal.

Clinical Reasoning, Problem Solving, and Long-Term Management	25%	Sophisticated reasoning with implications for physiotherapy, considers transferability and real-world application. Critically reflects on how evidence supports evidence-based practice in physiotherapy and future practice.	Strong reasoning with implications for physiotherapy, considers transferability and real-world application. Able to reflects on how evidence supports physiotherapy evidence-based practice	Competent reasoning; limited synthesis/depth in real world applications to physiotherapy. Demonstrates minor reflection	Basic reasoning; gaps in reasoning with implications for physiotherapy, considers transferability and real-world application No reflection on how evidence supports evidence-based practice	Inconsistent application; vague discussion on physiotherapy implications and real-world application No reflection on how evidence supports evidence-based learning.	Limited reasoning; weak problem-solving; vague discussion on physiotherapy implications and real-world application No reflection on how evidence supports evidence-based learning.	Poor reasoning; no understanding of physiotherapy or implications No discussion regarding evidence-based learning
Conclusion & Recommendations	10%	Sophisticated reasoning of quality of study, clear implications for physiotherapy; critical reflection on evidence supporting EBP now and for future practice in physiotherapy.	Clear, coherent summary; relevant recommendations with minor gaps. Identifies quality of study correctly. Provides rationale on research articles usefulness for practice.	Clear ideas but limited precision/depth in ability to determine quality of study and usefulness for practice.	General conclusions; limited relevance of usefulness for practice. Mentions quality of the study but in limited detail.	Adequate structure but limited coherence or relevance to physiotherapy practice. Comments on quality of the study.	Weak structure; unclear recommendations. No mention of physiotherapy practice or quality of article.	Very poor communication; no clear conclusions or recommendations. Does not mention quality of the study.
Presentation, Structure, & Referencing	10%	Highly organised work; excellent academic language; precise and consistent referencing. Uses the CASP appraisal tool succinctly throughout the assignment.	Very well structured; minor language or referencing issues. Uses the CASP appraisal tool with small errors throughout the assignment.	Clear presentation with some lapses in coherence or referencing consistency. Uses the CASP appraisal tool appropriately with a few errors throughout the assignment.	Adequate structure; language/ref issues affect readability. Uses the CASP appraisal tool throughout the assignment without much detail.	Adequate presentation but lacks logical flow; frequent referencing errors. Uses the CASP appraisal tool throughout the assignment without much detail or structure.	Weak structure; numerous errors; unclear presentation. Does not use the CASP appraisal tool throughout the assignment with no evidence of critical appraisal.	Poor structure; incoherent; lacks appropriate academic language. Does not use the CASP appraisal tool throughout the assignment with no evidence of critical appraisal.

PT5053 Quantitative – Marker Entry Sheet

Section	Marks Available	Awarded
Introduction	5	
Methods: Study Design	10	
Sampling & Recruitment	10	
Data Collection Methods	10	
Data Analysis & Findings	10	
Strengths, Limitations & Trustworthiness	10	
Relevance to Physiotherapy Practice	25	
Conclusion / Recommendations	10	
References & Quality of Writing	10	
Total	100	/100

PT5053 Qualitative – Marker Entry Sheet

Section	Marks Available	Awarded
Introduction	5	
Methods: Study Design	10	
Sampling & Recruitment	10	
Data Collection Methods	10	
Data Analysis & Findings	10	
Strengths, Limitations & Trustworthiness	10	
Relevance to Physiotherapy Practice	25	
Conclusion / Recommendations	10	
References & Quality of Writing	10	
Total	100	/100